Cervical Screening Test (Speculum) — OSCE Checklist

Deakin MD2 · ELC8 · Clinician-collected CST

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Prepare and consent → Equipment → Position → Inspect → Speculum → Sample → Remove → Close**. *Practised on part-task trainers — speak to the model as you would to a patient.*

1. Before the patient is on the couch

☐	Why a pelvic examination is needed; what it involves (speculum, CST, swabs, bimanual) and the position
☐	Past experience of pelvic examinations
☐	Obstetric history: children? vaginal births? (guides speculum size)
☐	Menstrual history: LMP; possibility of pregnancy
☐	Verbal consent; "Just say stop and I will"
☐	Offer a chaperone (advocate for patient and doctor; often a nurse who assists)
☐	Offer to empty bladder first
☐	Timing*: ideally ~2 weeks after day 1 of last period (if menstruating)

2. Equipment

☐	Gloves	Cushion / folded blanket to raise pelvis	Manoeuvrable light
☐	Lubricant	Range of speculum sizes	Swabs if needed
☐	Cytobrush + liquid-based vial (ThinPrep)	Single-use mat (bluey)	Sanitary pad and tissues
☐	Chaperone present	Vial labelled and form completed †	

3. Procedure

☐	Gather equipment; hand hygiene; gloves
☐	Patient undresses from waist down behind curtain and covers with blanket
☐	Enter with permission
☐	Position: supine, knees drawn up, heels together, let knees fall apart (do **not** push knees apart)
☐	Expose waist down only when ready; adjust light
☐	Inspect external genitalia: vulval skin, discharge, lesions, prolapse †
☐	Warn the patient; part labia with non-dominant hand
☐	Insert lubricated speculum with blades sideways, rotating during insertion as patient breathes out; aim towards sacrum, not head
☐	Gently open blades until cervix is visualised
☐	Fix speculum open; inspect cervix: os, ectropion, polyps, discharge, lesions †
☐	Insert cytobrush into the os and rotate 2–3 times
☐	Remove brush and agitate well in the ThinPrep liquid
☐	Loosen speculum and remove gently as patient breathes out (watch vaginal walls †)
☐	Cover patient with blanket; tell them the procedure is complete
☐	Privacy to dress; tissues and bin available

4. Closing

☐	Explain how and when results will be received; follow-up plan
☐	Invite questions; check comfort
☐	Document consent, chaperone, findings, tests taken, follow-up

5. Quick reference

- Bimanual examination is **not** recommended routinely in asymptomatic people attending for screening.
- Australian NCSP: HPV-based CST every 5 years for people with a cervix aged 25–74; self-collection is an option for everyone †.
- Clinician collection (HPV + liquid-based cytology) is required for symptoms (PCB, IMB, PMB, persistent unexplained discharge) and for AIS surveillance.
- Over 70% of Australians diagnosed with cervical cancer were under-screened or never screened.
- Pitfalls: inserting blades vertically, aiming upwards, opening blades before full insertion, closing blades on the cervix, exposing the patient before they're ready.